

Discharge Summary

Patient:	Elisabeth Braun
Date of Birth:	02/19/1956
Admission Date:	12/02/2024
Discharge Date:	12/11/2024
Attending Physician:	Dr. Katrin Fischer, MD

Dear Colleagues,

Ms. Elisabeth Braun, DOB 02/19/1956, was admitted on 12/02/2024 to initiate concurrent platinum-based chemoradiation for locally advanced cervical carcinoma. She presented 5 weeks prior with vaginal bleeding and pelvic pain. Biopsy confirmed squamous cell carcinoma. MRI: 5.2 cm cervical tumour extending bilaterally into the parametrium with left pelvic lymph node involvement. No distant metastases.

Primary Diagnosis: Cervical carcinoma (C53.9) — Tumor staging: T2b N1 M0

Laboratory values:

Parameter	Value	Reference
HbA1c	5.9%	< 6.5%
Hb	10.8 g/dL	12.0–16.0 g/dL
Creatinine	0.7 mg/dL	0.5–0.9 mg/dL

Past Medical History: Mild type 2 diabetes on metformin 500 mg once daily and diet modification (HbA1c 5.9%). Mild hypertension on perindopril 5 mg. No prior malignancy. Non-smoker, no alcohol. Primary school teacher, lives with long-term partner. Three adult children from prior relationship.

Fertility Discussion: A detailed fertility and ovarian function discussion was conducted with Ms. Braun and her partner prior to initiation of treatment. Given bilateral parametrial involvement requiring full pelvic radiotherapy, fertility-sparing approaches were not feasible. The permanent impact on fertility and the likelihood of early menopause were discussed fully. Oophoropexy was considered but not appropriate given lymph node involvement. Ms. Braun acknowledged

understanding and expressed agreement to proceed.

Nutritional Assessment: Ms. Braun had lost 4 kg in the preceding 6 weeks. BMI 21.8 kg/m². Nutritional risk score 2 — dietitian consultation obtained. High-calorie oral nutritional supplementation initiated at 2 × 200 mL per day.

Hospital Course: Radiotherapy planning CT performed day 2 with fiducial markers and planning approved. Weekly carboplatin AUC 2 initiated day 3 as radiosensitiser. First dose well tolerated — mild grade 1 nausea managed with oral ondansetron. No significant haematological toxicity on day 7 bloods. Brachytherapy planning discussed, first insertion scheduled for week 5 of external beam radiotherapy.

Psychosocial Support: The CNS specialist addressed the psychological impact of the diagnosis, treatment side effects, and long-term body image in two individual sessions. Ms. Braun reported significant anxiety regarding treatment but expressed resilience. She was referred to the oncology psychologist for ongoing support. Her partner attended one session and received separate carer support information.

Medication at Discharge: Metformin 500 mg, perindopril 5 mg, ondansetron 8 mg as needed, high-calorie nutritional supplements twice daily.

Follow-up: Radiation oncology day 8 for fraction 2. Gynecological oncology in 4 weeks. Weekly CBC during chemoradiation. Dietitian telephone review in 2 weeks.

Radiotherapy Planning Technical Details:

The radiotherapy planning CT was acquired with the patient in a supine position using a vacuum-bag immobilisation device. A full bladder protocol was employed to displace small bowel from the treatment field. Target volumes were defined in accordance with GEC-ESTRO guidelines. The gross tumour volume encompassed the primary cervical tumour and involved left pelvic lymph nodes as defined on MRI and PET-CT. The clinical target volume included the cervix, upper vagina, bilateral parametria, and pelvic lymph node stations at risk. Organs at risk — bladder, rectum, sigmoid colon, and loops of small bowel — were contoured and dose-volume histogram constraints applied. The plan was approved at the weekly radiotherapy planning meeting.

Side Effect Management and Patient Preparation:

Ms. Braun received detailed written and verbal information on the expected side effects of pelvic chemoradiation: acute effects (diarrhoea, urinary frequency and urgency, fatigue, perineal skin erythema, nausea) and longer-term effects (bowel and bladder dysfunction, sexual dysfunction, lymphoedema, early menopause). She was referred to the pelvic floor physiotherapist for pre-treatment assessment. A daily skincare regimen for the perineal area was provided, along with a low-residue dietary plan for use during radiotherapy to minimise bowel toxicity. The clinical nurse specialist will be available by telephone throughout the radiotherapy course for symptom management support.

Dr. Katrin Fischer, MD

Senior Physician, Gynecological Oncology